Is it necessary to remove tubes to prevent ovarian carcinoma?

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Disclosure of Conflict of Interest

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	Non relevant in this presentation

1. The facts: from tube to ovary
2. The clinical attitude
3. The technique
4. The limits

1. **The facts: from tube to ovary**

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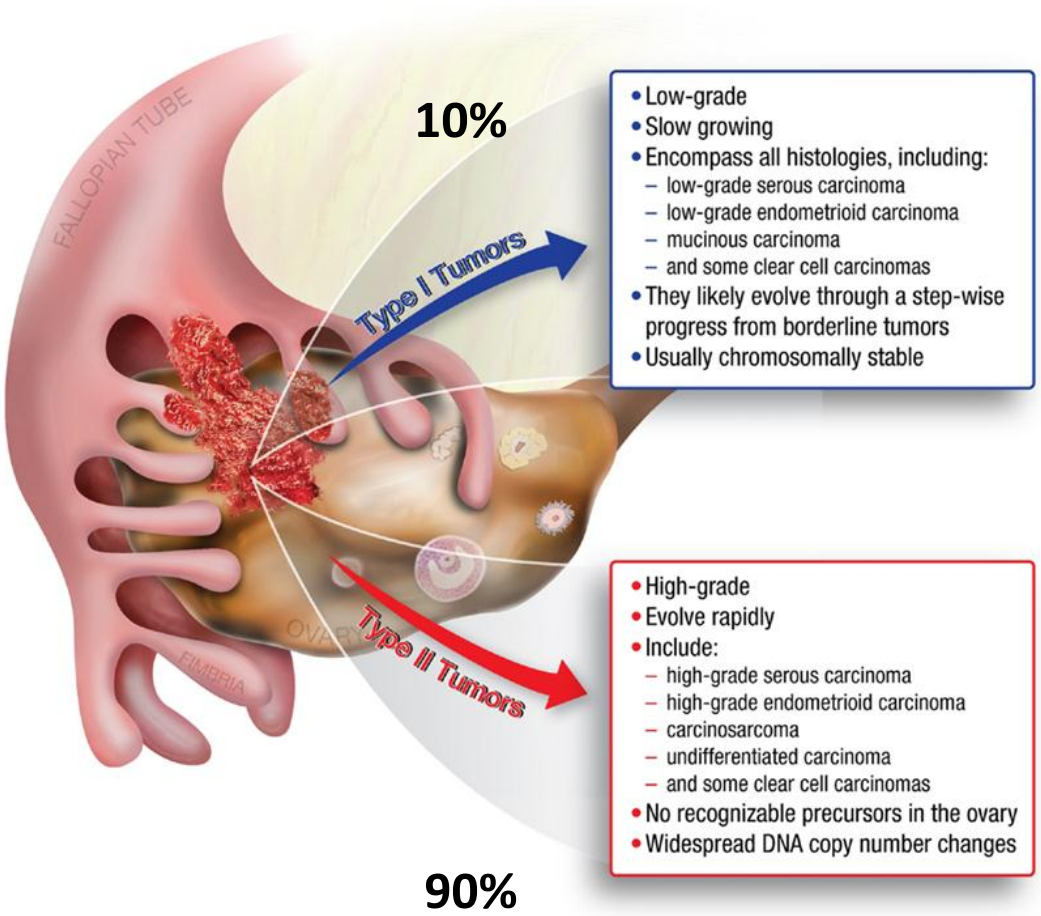
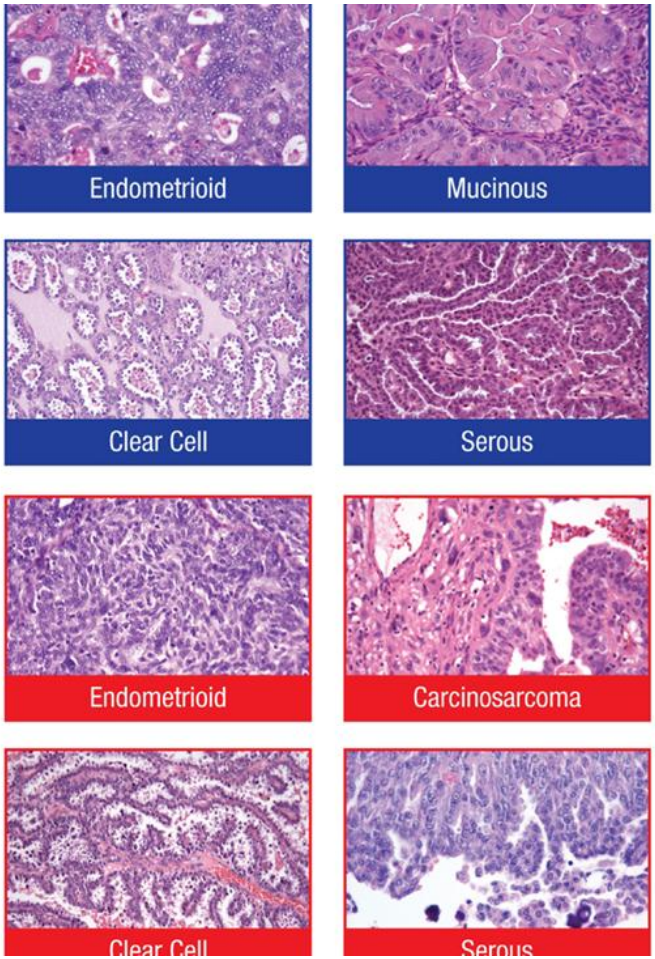
Ovarian tumors



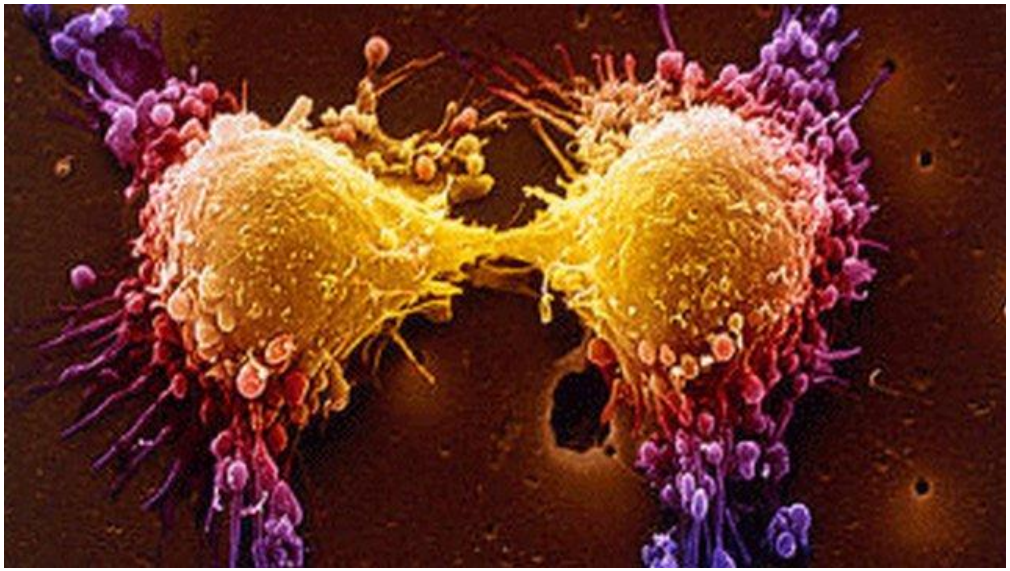


Front. Oncol., 26 August 2013.00217

Paul Michael Jones and Ronny Drapkin*

Tube and ovarian carcinoma

- Relationship between ovarian serous carcinoma and tubal lesions



Is the ovarian carcinoma originated from the tube?

- 55% of ovarian carcinoma are epithelial tumors
- 54% of these are serous
- Serous carcinoma=cells close to those of the tubal epithelium
- High grade serous ovarian cancer have frequently (3/5)a molecular p53mutation
- That is the carcinoma which is the more frequent in BCRA mutation.

- Int J Gynecol Pathol. 2011 Sep;30(5):417-24.**p53 signature and serous tubal in-situ carcinoma in cases of primary tubal and peritoneal carcinomas and serous borderline tumors of the ovary.Leonhardt K, Einenkel J, Sohr S, Engeland K, Horn LC.**

- Protocole SEE-FIM :Sectionning and Extensively Examinating the Fimbria End.

Systematic SEE FIM protocol in patients BCRA having had adnexectomy have shown
An important proportion of STIC (serous tubal intra epithelial carcinoma)
With intense expression of protein p53
These lesions are mostly located in the fimbria end

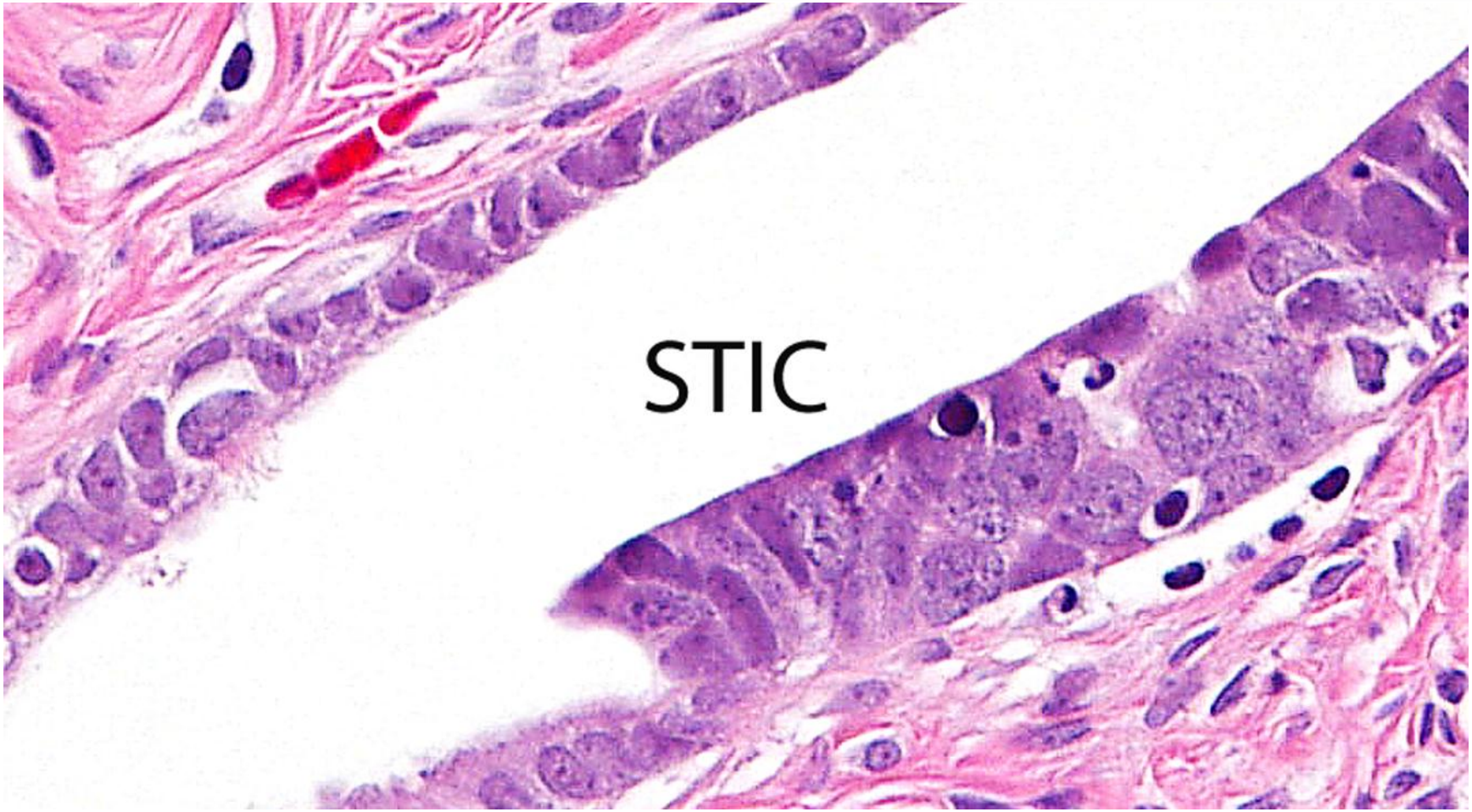
➔STIC= Serous Tubal Intra epithelial Carcinoma

Patient with BCRA mutation

- Meta analysis:341 patients
- Prophylactic annexectomy
- ➔3,8% serous carcinoma
- If SEE protocol is applied (tube+ovary)➔17%
- In which 57% are STIC (fimbria)

- Powell CB, J.Clin.Oncol. 2006,23, 127-132

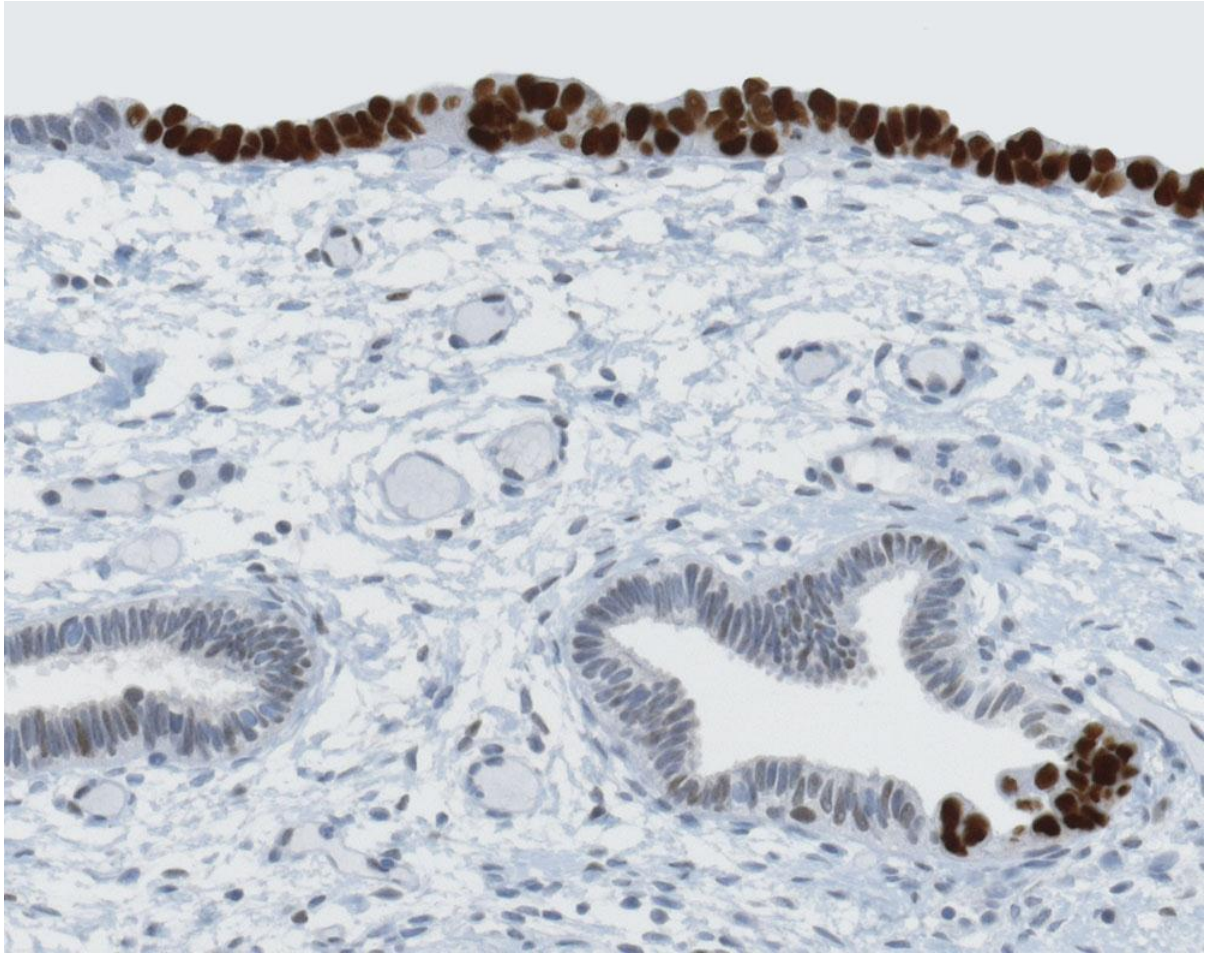
Serous tubal intra epithelial carcinoma



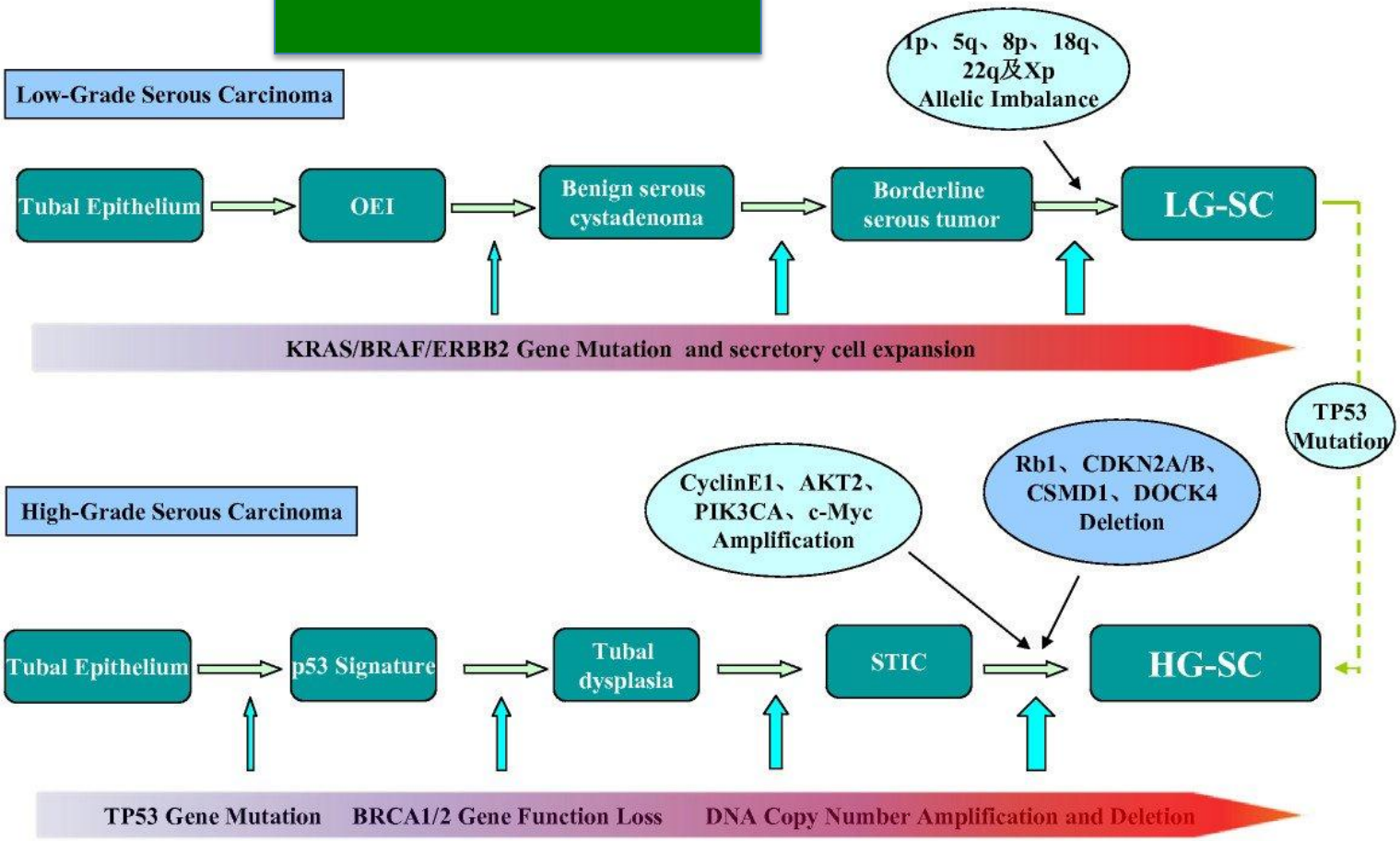
(scat=serous carcinogenesis in the tube,Chen 2010)

Preliminary stage	step 1	step 2	step 3
Latent precursor	Latent precursor		
SCOUT(secre tory cell outgrowth)	Signature p53	TILT (tubal intraepithelial lesion in transition)	STIC (serous tubal intraepithelial carcinoma)
alteration DNA	Alteration DNA Mutation p53	Alteration DNA Mutationp53 Stratifications	Idem+ Nuclear cell atypia

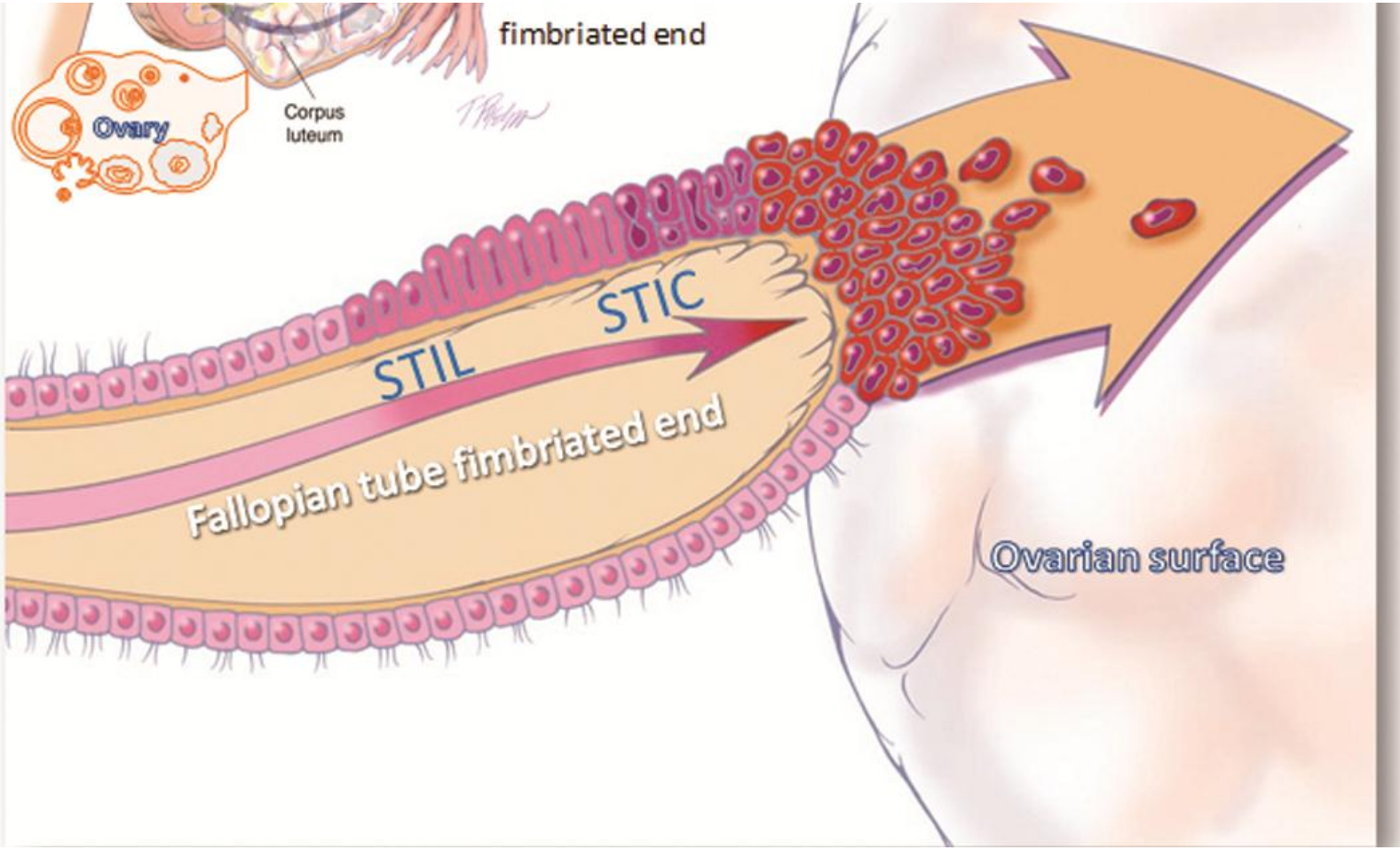
P53 signature



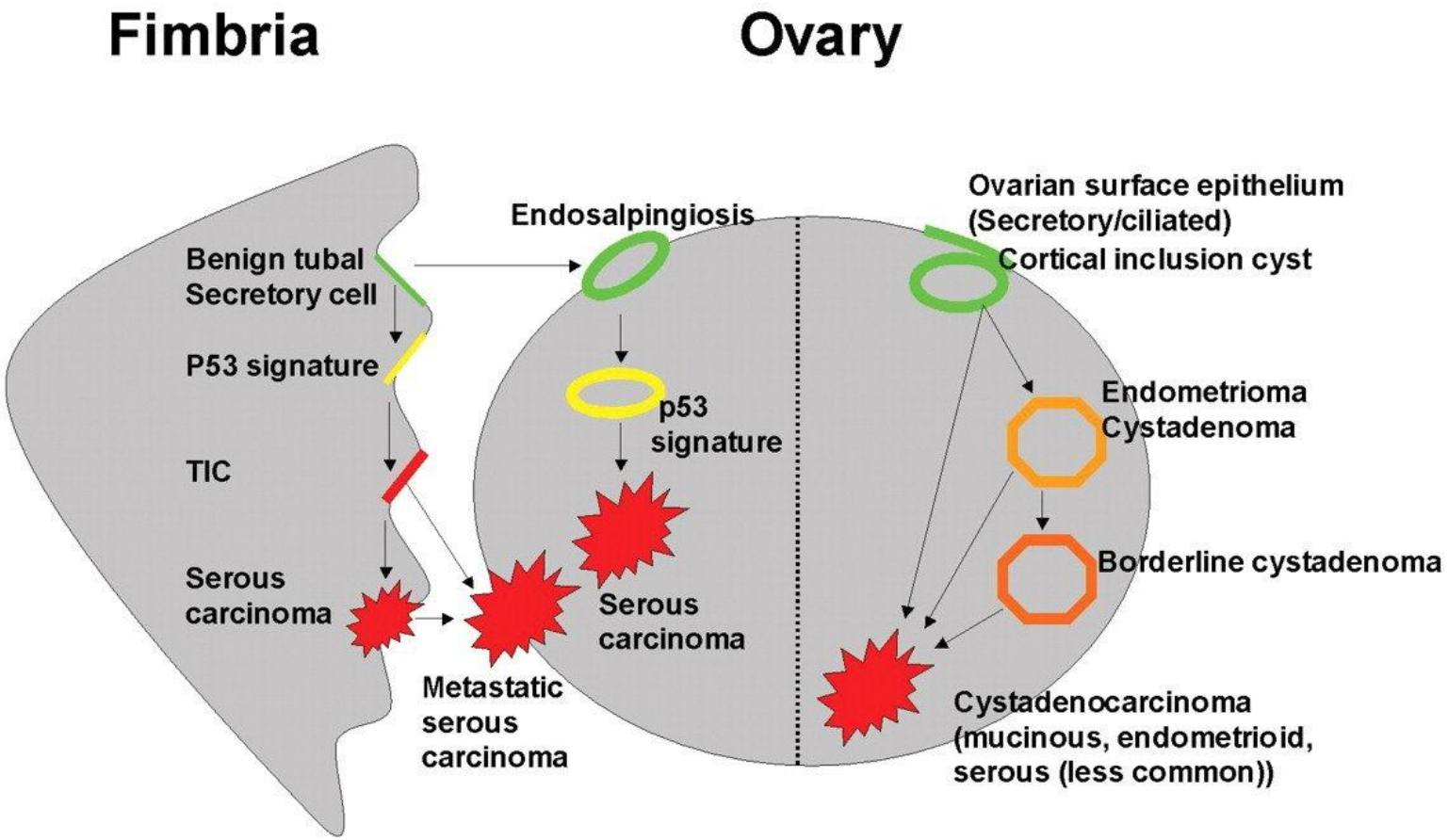
CARCINOGENESIS



Li *et al. Journal of Hematology & Oncology*
2012 **5**:8 doi:10.1186/1756-8722-5-8


fimbriated end
Ovary
Corpus luteum
STIC
STIL
Fallopian tube fimbriated end
Ovarian surface

From tube to ovary



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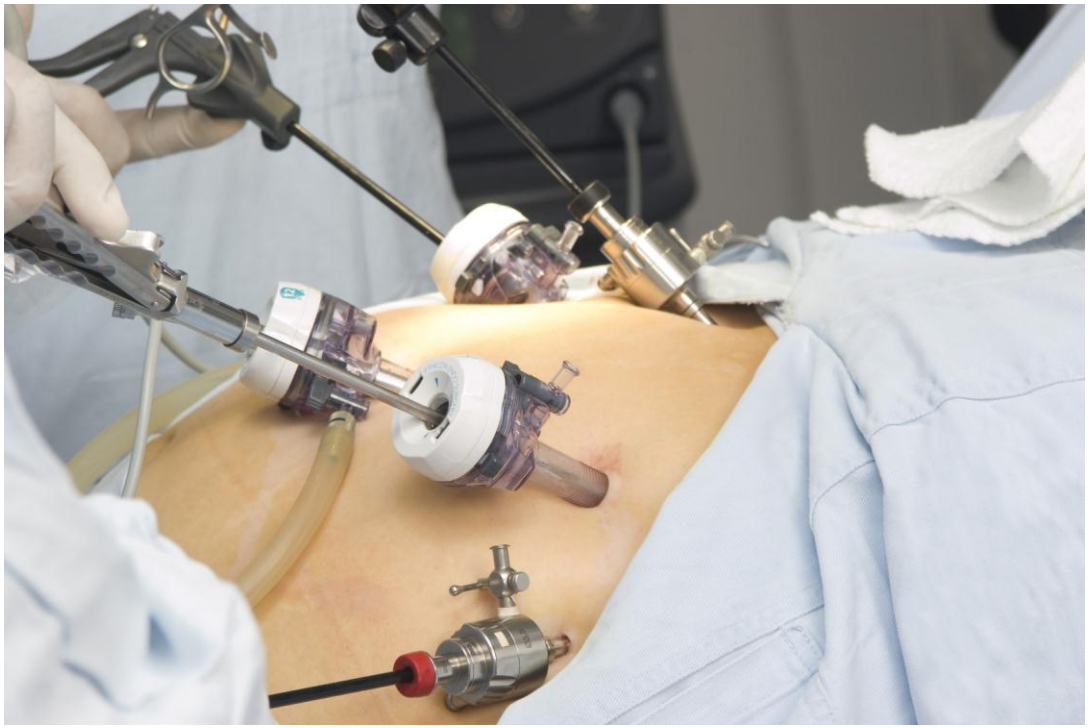
Risk factors of STIC

Risk factors: **tubal inflammation** (chronic salpingitis)

Inflammation and menstrual reflux induce liberation of cytokines (interleukine8, TNF alpha) which may damage DNA and lead to STIC

(on the **contrary protect effect of pregnancy and oestro progestative contraception**)

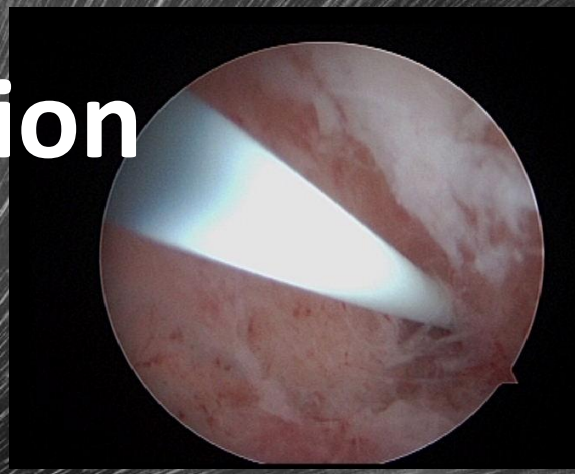
When to do salpingectomy?:Clinical proposal



In case of Hysterectomy

- Under the age of 50=systematic salpingectomy should be proposed
- Above the age of 50= classical attiitude =annexectomy

In case of sterilization



- Salpingectomy only if:

-Essure® not available

-Patient over 40

-after information and consent

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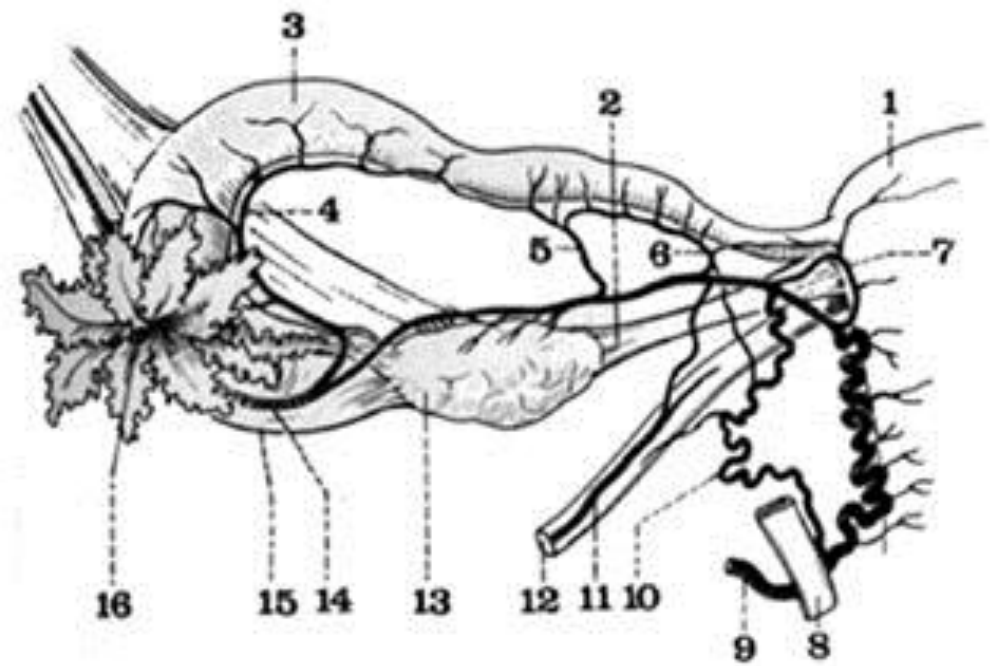
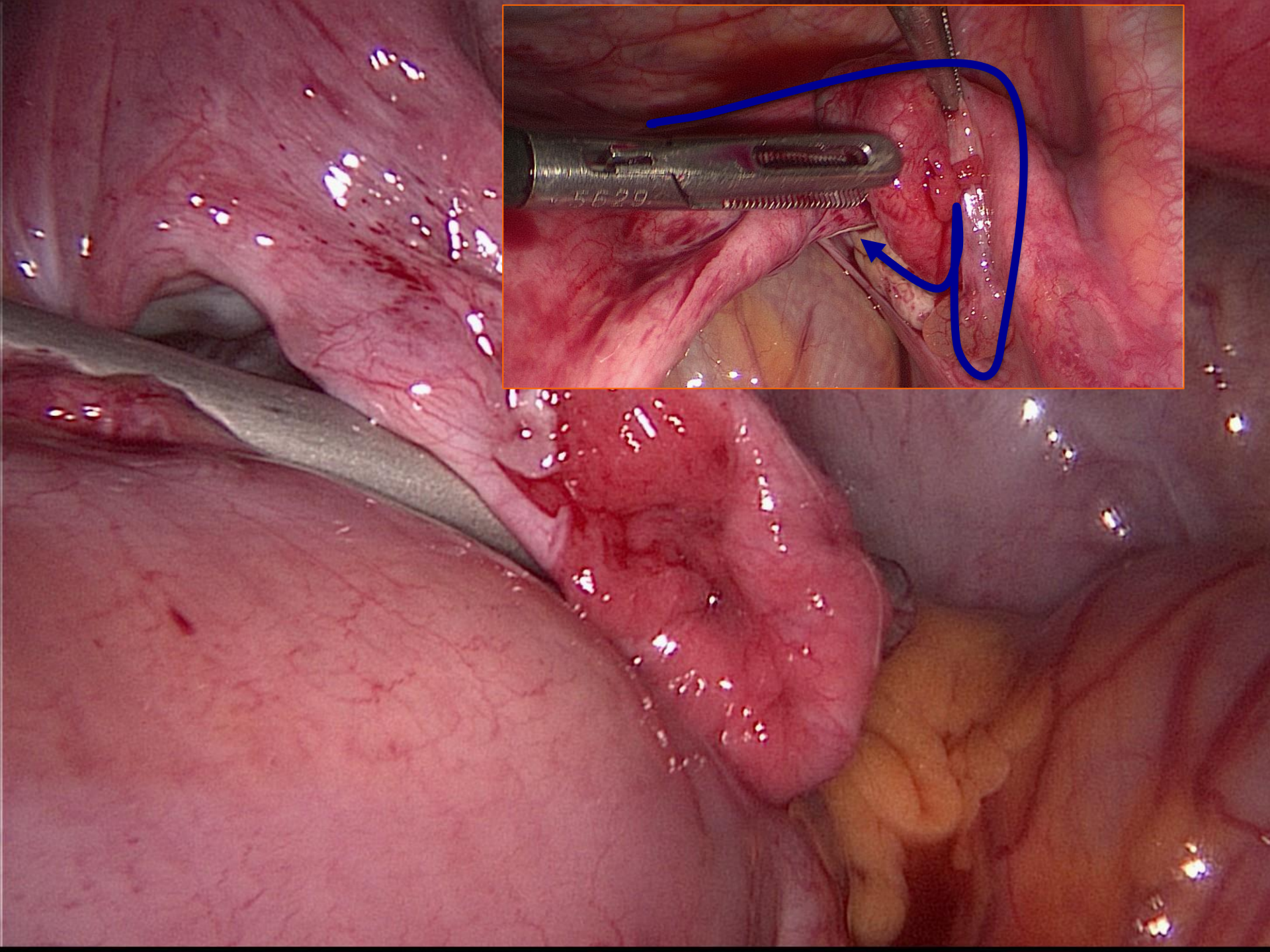
Technique of salpingectomy

- Stay as close as possible of the tube (to respect ovarian blood supply)
- Total(especially all the fimbria)





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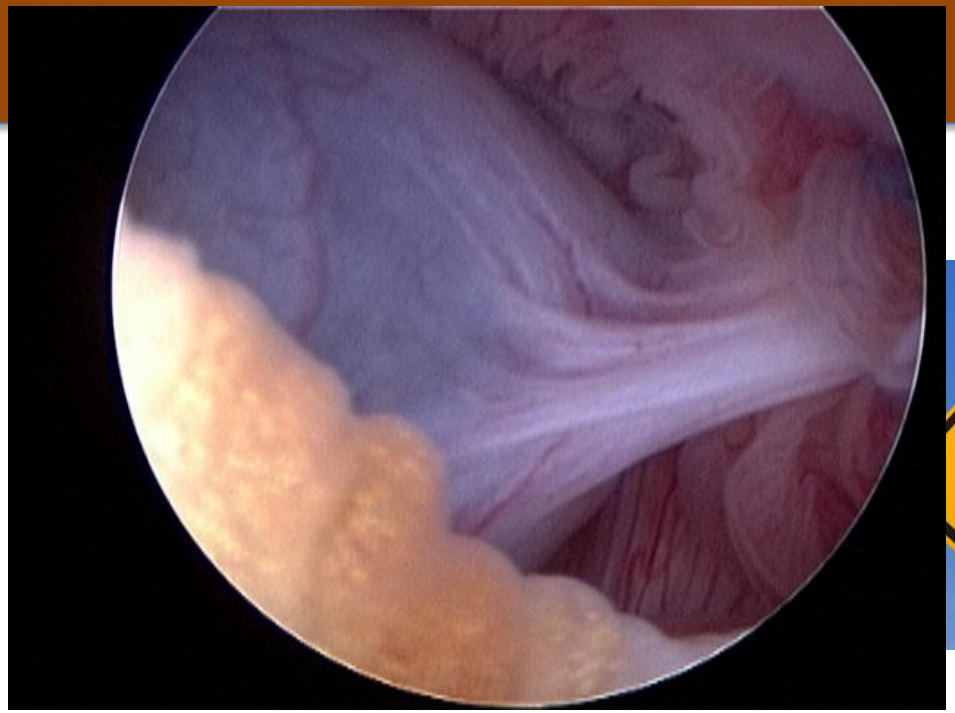
Clinical limits

- In BCRA patients evidence is still insuficient to propose salpingectomy instead of annexectomy



-More and more evidence of Relationship between STIC and serous ovarian cancer should lead to prevent the disease

-But:

Real impact on ovarian cancer prevention will take time to be proven

Thank you!